

Media Information

9:46 AM

Fax Services

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Atlantic Medical Imaging - Synapse EIS

**Accurate answers. Fast.**

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Exam Date: 03/21/2025 03:21 PM

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cc
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Addendum 1

PATHOLOGY ADDENDUM CREATED 4 AM

PROCEDURE:

1. Right breast ultrasound-guided core needle biopsy of right breast mass at 10:00 approximate 10 cm from nipple (tophat clip)

Pathology demonstrates:

"Benign breast tissue with focally prominent stromal fibrosis."

See complete pathology report, which has been faxed to the ordering clinicians office.

MANAGEMENT:

1. The radiological imaging and pathology results are benign and concordant.
2. Patient was initially scheduled for breast MRI 8/14/2025. However, on phone conversation with her on at 1015 she stated insurance authorization has been delayed and MRI had been rescheduled for 9/9/2025. At this time plan is to follow-up MRI findings to see if there is a region of enhancement anterior to the biopsy site by approximately 2 cm, corresponding to the region of questioned asymmetry on diagnostic mammography Pending MRI results decision will be made for MRI versus stereotactic biopsy versus short-term follow-up.

The results and recommendations for follow-up were discussed by myself, Dr. Clinton Veselis, with and acknowledged by the patient understanding and all questions were answered to her satisfaction. Patient states the biopsy site is healing well. The results were also sent to the patient's clinician.

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If you are the referring physician (609)650-0004 to reach a radiologist.

Date Of Signature: 8/14/2025 10:18 AM

Transcribed By: Veselis, M.D., Clinton

Transcribed Date: 8/14/2025 10:18 AM

Electronically Signed by: Veselis, M.D., Clinton

Signed Date: 8/14/2025 10:18 AM

ULTRASOUND-GUIDED CORE BIOPSY RIGHT AND RIGHT POST-PROCEDURE MAMMOGRAM FOR
MARKER PLACEMENT: 8/3/2025 3:31 PM

INDICATION: 53 years old, with irregular hypoechoic mass at 10:00 centimeters from the nipple with right breast asymmetry on mammography.

COMPARISON: 9/12/2023

Findings: The procedure including risks, benefits and alternatives were explained to the patient. The discussion of risks included but was not limited to pain, bleeding/bruising, and infection. The patient was given the opportunity to ask questions and acknowledged understanding. A written informed consent was obtained.

The patient was brought into the ultrasound suite for biopsy. Multiple sonographic images of the right breast irregular hypoechoic mass at 10:00 10 cm from the nipple were obtained. A time out procedure was performed to verify the patient's identity and location of the biopsy.

The skin was antiseptically cleansed and draped. Local anesthesia was achieved to the skin puncture site using a 1% lidocaine solution and a 1% lidocaine solution with epinephrine deeper surrounding the biopsy site. A skin nick was made with an 11 blade.

Under realtime sonographic guidance, multiple core biopsies were obtained using a Bard, 14-gauge, Marquee.

The needle was seen to pass through the lesion during the procedure. After assuring adequate sampling, a biopsy marker clip in the shape of a tophat was placed.

Hemostasis was achieved with manual compression. Sterile dressing was applied after compression was held. No complications occurred.

All cores were placed in formalin and submitted to pathology for histologic analysis.

Right POST-PROCEDURE MAMMOGRAM FOR MARKER PLACEMENT:

BREAST COMPOSITION: The breasts are heterogeneously dense, which may obscure small masses (Category C).

A post-procedure two-view mammogram shows biopsy marker clip is posterior to the previously reported right breast mammographic asymmetry by approximately 2 cm. No

Patient was given verbal and written postprocedure instructions. The patient experienced no

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IMPRESSION

1. Technically successful ultrasound-guided core needle biopsy of right breast mass at 10:00 approximately 10 cm from the nipple (tophat clip). Post procedure mammogram demonstrates the biopsy clip posterior to the questioned mammographic asymmetry. Patient [REDACTED] had with the patient that right breast ultrasound-guided biopsy correctly targeted ultrasound findings from prior diagnostic ultrasound 8/4/2025, however, right breast asymmetry may be distinct from right breast ultrasound findings and therefore scheduled right breast MRI on [REDACTED] is still recommended as this mammographic asymmetry may represent a separate finding from ultrasound finding. Decision to perform stereotactic/tomographic versus MRI guided biopsy will be dependent upon both MRI and pathologic results.
2. Right post-procedure mammogram for marker placement.

Awaiting pathology report. An addendum to this report will be made with final pathology results.

In accordance with New Jersey law, the following statement must be included in all mammography reports, regardless of an individual's breast tissue density:

"Your mammogram may show that you have dense breast tissue as determined by the Breast Imaging Reporting and Data System established by the American College of Radiology. Dense breast tissue is very common and is not abnormal. However, in some cases, dense breast tissue can make it harder to find cancer on a mammogram and may also be associated with a risk factor for breast cancer. Discuss this and other risks for breast cancer that pertain to your personal medical history with your health care provider. A report of your results was sent to your health care provider. [REDACTED] at the website of the American College of Radiology, www.acr.org."

If you are the referring physician [REDACTED] (609)650-0004 to reach a radiologist.

Date Of Signature [REDACTED] 4:36 PM

Report Electronically Signed by: Vesels, M.D., Clinton
Report Electronically Signed on: [REDACTED] 04:36 PM

Patient [REDACTED]
Patient [REDACTED]

[REDACTED] BX w/Clip | [REDACTED]

aniris/WebForms/Patient/PatientId [REDACTED]

Document Information

Other: Outside Pathology

PATH ADDENDUM

00:00

Attached To:

OUTSIDE PATHOLOGY ORDER

Orders Only on with Historical Provider,

Source Information

Priscilla P. Hazel | Msk Pas